

OCCUPATIONAL ENGLISH TEST

WRITING SUB-TEST: MEDICINE

**TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES**

Read the case notes below and complete the writing task which follows.

Notes:

You are a general practitioner. Ms Sarah Day, 20 years old, has been attending your practice since her early childhood.

Patient: Ms Sarah Day, D.O.B. 29.07.1997

Past medical history: Jul 2001: varicella
Apr 2002: measles
Jan 2004: fractured ulna
Jun 2006: URTI
Sep 2008: plantar warts
Dec 2011: dysmenorrhoea
Apr 2013: teenage acne
Aug 2014: pre-menstrual syndrome
Jun 2015: exam-related anxiety
Nov 2016: oro-facial herpes simplex virus

Social background: University student - not happy with course of study
Part-time job - McDonald's
Irregular hours, 5-8 hrs sleep/night
Smokes, drinks moderately
Lives at home with parents, boyfriend for 7 months

Medications: Oral contraceptive pill (prescribed April 2013) cyproterone acetate/ethinylestradiol - mane

06.12.17 Pt presented with mother complaining of unilateral headache - occipital, temporal extending to vertex, dizziness/loss of balance, with nausea and anxiety. Visual disturbances. Better when lying down. Symptoms not affected by red wine, alcohol, chocolate, cheese, sunlight. Not related to menstrual cycle or stress.

Pt reports workload stress, pressure of assignments & exams.

Episodes: x2 in past 2 weeks.

Pain: Steady, not throbbing.

Onset: Rapid
Aura
Symptoms last 1-2 days (severe for several hours).
No family history of headaches/migraines.

Examination: No abnormal neurological signs.

Treatment: Education: rest/relaxation/meditation, adequate sleep, regular eating.
During attack: sleep, dark room, ice.
Metoclopramide 10 mg - orally -> 20 mins later paracetamol 1 g - 4 hrly (max 4 g/24 hrs).

04.01.18 Attacks more frequent. Prescribed medications ineffective.
Pain felt in neck.
Anxiety becoming worse.
Dizziness during attacks.
Vomited on one occasion.
Mother reports Pt becoming afraid to leave house in case attack occurs.
No identifiable triggers.
Dark room, ice - no effect.

Treatment: Eletriptan 40 mg - orally on attack.
Ibuprofen 400 mg - 6 hrly (max t.d.s.).

31.01.18 Pt complained of drowsiness and diarrhoea since commenced on eletriptan.
No improvement in symptoms. Anxiety worse - Pt describes as 'panic' accompanying symptoms.
Mother concerned daughter becoming depressed: withdrawn, housebound, losing interest in activities & boyfriend, oversleeping, comfort eating -> weight gain.

B.P 120/80

Treatment: Amitriptyline 25 mg - b.d.

24.02.18 Pt presented alone. Complains of numbness and tingling (paraesthesia) in fingers 4 & 5 left hand.
No improvement in symptoms.
Pt reported car accident Jun 2017. Not previously disclosed as wishes to keep from mother.
Sustained 'whiplash injury'.
No treatment sought at time.
Referral to neurologist, for investigation and management.

Writing Task:

Using the information in the case notes, write a letter of referral to Dr Robert Edwards, a neurologist. Outline Ms Day's recent history and request further investigations and management. Address the letter to Dr Robert Edwards, Rushford Hospital, Long Gully Road, Littletown.

In your answer:

- Expand the relevant notes into complete sentences
- Do not use note form
- Use letter format

The body of the letter should be approximately 180-200 words.